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A systematic scoping review of peer support interventions in integrated primary youth mental health care

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Abstract

Peer support, defined as the social and emotional support offered and received by individuals with a shared experience of mental health difficulties, is gaining popularity in youth mental health settings. This systematic scoping review aimed to collate and synthesise the evidence on key aspects of peer support interventions within integrated youth services and educational settings. Specifically, it synthesised evidence on the (1) assessed mental health outcomes in peer support interventions, (2) key characteristics and associated roles of peer support workers (PSWs) and (3) barriers and facilitators to implementation. A search of peer reviewed articles from January 2005 to June 2022 across five electronic databases (PsychINFO, Pubmed, Scopus, ERIC and CINAHL) was conducted. A total of 15 studies retrieved in the search met the inclusion criteria and were included in the review. This review supports previous research indicating that peer support has potential for improving recovery related outcomes. While a variety of interventions and PSW roles were reported, studies could be strengthened by providing more in-depth information on intervention content. Examples of barriers to implementation included staff concerns around confidentiality of

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peer support relationships as well as PSWs' confidence in their roles. Facilitators included positive support from staff members and role clarity.

KEYWORDS

early intervention, educational settings, integrated primary youth mental health care, intervention, mental health, peer support, youth

1 | BACKGROUND

The term *youth* encompasses the developmental phases of adolescence and emerging adulthood, and spans the early teenage years to the mid-twenties (Sawyer et al., 2018). During this period, young people experience significant biological, psychological, and social changes (Birchwood & Singh, 2013) which can place them at potential risk of psychological vulnerability. Mental health difficulties are prevalent, with anxiety and depression among the top five causes of ill health in ages 15–29 (The Lancet, 2022). Conversely, young people often do not or cannot access adequate mental health support (McMahon et al., 2019). Several micro and macro level barriers can prevent help seeking, including financial and structural barriers, or perceived stigma around mental health (Radez et al., 2022; Salaheddin & Mason, 2016).

To address these barriers and increase access to care, an international movement has recently emerged with the aim of providing youth specific services (McGorry et al., 2022; McMahon et al., 2019; O'Reilly et al., 2022). *Integrated primary youth mental health care* models have been designed to acknowledge the complex psychological and social challenges young people experience (McGorry et al., 2022). Two prominent settings in which this model has been adopted are specialist integrated youth services (IYs) and school and university mental health services (Duffy et al., 2019; Hetrick et al., 2017; Kutcher & Wei, 2020; McGorry et al., 2022). Although the two settings differ in their structure, both are recognised as important locations in which young people can access localised, community based, early intervention mental health support (Duffy et al., 2019; Kutcher & Wei, 2020; McGorry et al., 2022).

IYs are identified by several characteristics. Most notably, they are accessible in terms of location and cost, they are located in highly visible and community spaces, they offer non-judgemental and non-stigmatising support, and they involve young people in service design and delivery (Hetrick et al., 2017; McGorry et al., 2022; Settapani et al., 2019). Notable examples of these services include Jigsaw, Ireland, formally Headstrong (established 2006), ACCESS Open Minds, Canada (established 2014) and Maisons des Adolescents, France (established 1999).

Similarly, educational settings have been recognised as ideal locations to address students' mental health needs given that they constitute single settings in which a large proportion of young people can be reached (Kutcher & Wei, 2020; McMahon et al., 2019). Although not as rigorously defined as IYs, school and university services are beginning to adopt this model by implementing a range of accessible, youth friendly and evidence-based mental health interventions within their community (Duffy et al., 2019; Kutcher & Wei, 2020). Examples of this approach can be seen in tertiary education in the United Kingdom (Newton et al., 2016) and Ireland (Armstrong-Astley et al., 2022; Hill et al., 2020), as well as secondary education in Canada (Kutcher & Wei, 2020) and Australia (Anderson & Cooke, & Zbukvic, 2020).

To ensure integrated primary youth mental health (YMH) care models are developmentally appropriate and person centred, it is recommended that young people are at the heart of service delivery (Halsall et al., 2021; O'Reilly et al., 2022; Rickwood et al., 2019; Zbukvic et al., 2020). One component of youth participation that is gaining attention is the involvement of young people through peer support (Anderson & Cooke, & Zbukvic, 2020; Hennessy et al., 2022; Osborn et al., 2022).

Peer support, in the context of mental health, is defined as the social and emotional support offered and received by people who have a lived experience of a mental health difficulty, with the aim of bringing about social or personal change (Davidson et al., 2005). Solomon (2004) outlines six categories of peer support interventions: (1) peer support groups, (2) internet support groups, (3) peer-delivered services, (4) peer-operated services (planned and delivered by peer support workers [PSW]), (5) peer partnerships (a sponsoring organisation shares administration and governance, but primary control is with peers), and (6) peer employees (individuals who identify as having a mental illness who are hired into unique peer positions or who are employed to serve traditional mental health positions).

Interest in peer support interventions is growing within IYSs and educational settings for several reasons (Halsall et al., 2021; Mantzios, 2019). Firstly, peer relationships become increasingly important during youth which is a peak developmental period for the emergence of mental health difficulties (Simmons et al., 2023). Peers are a natural resource in the help seeking process, with young people citing friends as the most common source of mental health information and support (Dooley et al., 2019). Additionally, the principles of peer support differ from traditional client-clinician relationships in that they are purposeful, where both young people (or groups of people) form a mutual connection, share knowledge and viewpoints, and support and challenge one another to move forward (Mead, 2005; Repper & Carter, 2011). As a result, peer support may be an entry way to mental health care for those hesitant to seek help from a professional (Simmons et al., 2023).

Emerging evidence suggests that this interest is warranted. Recent reviews have indicated that researchers are beginning to recognise peer support as a whole of life, rather than illness approach to mental health care (Gillard, 2019) and are measuring mental health outcomes accordingly. Several reviews of the adult peer support literature have identified improvements in quality of life, empowerment, hope and recovery (Bellamy et al., 2017; Chinman et al., 2014; Lloyd-Evans et al., 2014; White et al., 2020). Similar findings have been noted in reviews of formal and informal peer support in a variety of YMH and community settings such as improved psychological wellbeing, empowerment, self-esteem, and social functioning (Gopalan et al., 2017; Richard et al., 2022). In educational settings, evidence suggests that peer support can increase self-confidence, self-esteem, self-management, hope, empowerment and reduce loneliness (Johnson et al., 2018; King & Fazel, 2021; White et al., 2020).

Overall, the available evidence on peer support for YMH is promising. However, much of this comes from reviews of peer support in community settings or secondary/tertiary mental health services (Gopalan et al., 2017; Richard et al., 2022; Simmons et al., 2023). A comprehensive overview of the reported changes in mental health outcomes is essential to confirm these benefits in integrated YMH care, and explore further potential benefits (Anderson & Zbukvic, 2020).

In addition to the understanding the impact of peer support on mental health, a key priority for research is identifying the features of PSWs involved in these interventions (King & Simmons, 2018). PSWs are predominately defined as people with experiences of mental health difficulties. In YMH settings, similarity in age is also often understood to be a requirement for the role (de Beer et al., 2022; Fava et al., 2020). Both lived experience and age can enhance the relationship between a young person and PSW by increasing the likelihood of shared experiences (Fava et al., 2020). However, although these characteristics are recommended, it is unclear whether services are recruiting PSWs based on these requirements.

It is also recommended that peer support is introduced to services systematically, such that PSWs are recruited with the above-mentioned characteristics in mind and are provided with opportunities for professional development. Adequate training and appropriate supervision are particularly important in youth settings, given the age and lack of employment experience of young people recruited into these roles (de Beer et al., 2022).

Furthermore, role requirements should be made clear to PSWs and staff working within a service (Collins et al., 2016). Lack of clarity can result in PSWs experiencing difficulties integrating with services (Collins et al., 2016); feeling tokenistic (Kilpatrick et al., 2017); or conversely, becoming overworked or burnt-out in their role (de Beer et al., 2022). de Beer et al. (2022) previously investigated roles of PSWs across youth settings,

including psychiatric care, community mental health services and youth offending services. They found that PSWs performed a variety of roles, including advocating for peers, planning treatment, educating peers and providing emotional support. In primary and community mental health care, Fava et al. (2020) suggests that PSWs provide face-to-face and/or group peer work, predominately focusing on shared experiences, recovery goals, and a broad focus on mental health. However, they also note that these roles can vary significantly from service to service. As such, it is unclear if there are consistent roles across integrated primary YMH care. Thus, while it is evident that there are potentially unique characteristics and role requirements for PSWs in these settings, no review to date has synthesised these data, which would be of value to services in the development of role requirements and training programmes.

Finally, it should be noted that successful peer support interventions do not solely rely on the relationship between a young person and PSW, but also on the organisation and the staff working within it (Hopkins et al., 2021). Previous reviews of peer support in adult mental health services suggest that several barriers can impede implementation of these interventions. These include staff concerns or confusion around the role of PSWs; inadequate recruitment processes, and lack of training and supervision (Hopkins et al., 2021; Ibrahim et al., 2020). Conversely, facilitators of peer support in adult settings include strong leadership, organisational readiness, rigorous recruitment strategies, and team integration (Kent, 2019; Mutschler et al., 2022; Zeng & McNamara, 2021). Although systematic reviews have examined implementation of adult peer support (Ibrahim et al., 2020; Mutschler et al., 2022; Zeng & McNamara, 2021), no review to date has examined the barriers and facilitators to peer support in YMH settings, despite this being a recognised priority for future research (Anderson & Zbukvic, 2020). Furthermore, Chinman et al. (2017) emphasise the need to examine the current literature on barriers and facilitators at the organisational level through implementation frameworks such as The Consolidated Framework for Implementation Research (CFIR). Such frameworks are valuable in understanding how and why a specific intervention is or is not implemented successfully (Damschroder et al., 2009).

1.1 | The present study

While interest in peer support for YMH care is growing, a substantial gap remains in our understanding of these interventions in the context of integrated primary YMH care, including IYSs and educational settings. The aim of this scoping review is to identify, collate and synthesise the available evidence on key aspects of peer support interventions within IYSs, and school and university settings. Specifically, this review aims to address the following research questions:

- (1) What mental health outcomes have been reported in studies of peer support interventions in integrated primary YMH care?
- (2) What are the core characteristics of PSWs and their subsequent roles in integrated primary YMH care?
- (3) What are the barriers and facilitators to implementing peer support interventions in integrated primary youth care mental health care?

2 | METHODS

A systematic scoping review methodology was employed to answer the research questions. Scoping reviews are a type of evidence synthesis that aim to systematically map the available research in a given area and to identify gaps in the field of research (Arksey & O'Malley, 2005; Munn et al., 2018, 2022). A research protocol was developed in accordance with the Preferred Reporting Items for Systematic Reviews and Meta-Analyses (PRISMA-ScR) extension for Scoping Reviews (Tricco et al., 2018) and registered on Open Science Framework;

<https://osf.io/fbq2n/>. The review was carried out in distinct phases, as described by Arksey and O'Malley (2005), including the development of the research questions; identification of potentially relevant studies; screening and selecting papers; charting the data from included papers; collating, summarising, and reporting the results; and consultation with stakeholders.

2.1 | Youth participation

A participatory approach was adopted to develop the research questions. During initial planning stages, a consultation was held with the Youth Research Council (YRC) at (YMH service). The YRC was comprised of a group of young people (aged 20–25; $n = 8$) with lived experiences of mental health difficulties and/or an interest in YMH. The lead author met the group via Zoom for a 1-h meeting and discussed the purpose of the review and initial research questions. The collaborative platform, Padlet was used to collate feedback from members. The YRC confirmed that the research questions were appropriate for capturing the available evidence on peer support in integrated primary YMH care.

2.2 | Search strategy

A search strategy was developed through the identification of terms used in reviews of a similar nature (e.g., Gopalan et al., 2017; Richard et al., 2022) and consultation with a specialist librarian. The strategy was informed by the PCC (Population of Interest, Concept, Context) model for scoping reviews (Peters et al., 2020). Terms related to youth (population), peer support (concept) and YMH services/schools and universities (context) were included in the search. Following preliminary assessment of electronic databases for their relevance and coverage of the topic literature, five databases were identified to carry out the electronic search: Psychinfo, Pubmed, ERIC, CINAHL and Scopus. These databases were searched for peer-reviewed articles between September 1st 2005 and June 2022 13th 2022. The full search string is presented in *Supporting Information Material*. To avoid missing relevant publications, backwards and forwards citation searching was conducted on studies selected for data extraction.

2.3 | Inclusion and exclusion criteria

Studies were included if they met the following criteria:

- (a) The studied intervention was aimed at young people between 12 and 25 years, or the majority of participants fell within this range.
- (b) The studied intervention was aimed at young people experiencing mild to moderate mental health difficulties.
- (c) The study involved a PSW(s) partially or fully delivering the intervention.
- (d) The studied intervention was delivered within an IYS or a secondary or tertiary educational setting.
- (e) The study was published in a peer reviewed publication.
- (f) The study was published in English, between 1st September 2005 and June 13th 2022.

For the purpose of this review, mild to moderate mental difficulties were defined as any mental, behavioural or emotional condition which do not require specialist intervention (e.g., inpatient care) and that are typically managed in primary care or go undiagnosed. The review excluded peer support interventions aimed at young people with serious mental illness, such as bipolar disorder, schizophrenia, psychotic conditions or any disorder resulting in serious functional impairment (National Institute of Youth Mental Health, 2023). Following an initial search of the

literature, a decision was made to limit the search from 2005. This decision was made for several reasons. Firstly, the vast majority of IYSs launched following the conception of the headspace model in 2006 (McGorry et al., 2022). Secondly, evidence suggests that schools and universities began to take a more integrated approach to mental health care within the last decade (Duffy et al., 2019). Thus, an increased focus on peer support in these settings is likely to have occurred post 2005.

In this review, studies were excluded if they: were literature, scoping or systematic reviews, study protocols, book chapters, dissertations, conference abstracts or editorials; examined informal peer support, for example peer support provided by friends; examined peer support for non-mental health outcomes, for example academic, social or occupational outcomes; did not include a PSW or were fully delivered by a mental health practitioner.

2.4 | Study selection

Search results from each database were exported to the reference management software Zotero (<https://www.zotero.org/>) and then exported to Covidence Systematic Review Software (<https://www.covidence.org/>) for title, abstract and full text screening. Title and abstracts of studies were screened independently by two reviewers (XX and XX). Studies selected for full text review were screened again by the two independent reviewers (XX and XX). As recommended by Levac et al. (2010) reviewers met regularly throughout this screening process to resolve any uncertainties. A third reviewer was consulted (XX), where a decision regarding inclusion could not be reached. Following full text review, citation searching and manual searching of previous reviews was completed by XX, and any potentially relevant articles were added to Covidence, and the process was followed through again.

2.5 | Data extraction and analysis

After study selection, the first reviewer (XX) conducted data extraction on all included articles. A second reviewer (XX) conducted extraction on 50% of studies to ensure consistency. The extraction tool was developed through an iterative process which consisted of consultation with the research team (XX & XX) and the completion of pilot extractions. Key information that was extracted included: (a) study characteristics (authors, title, date of publication, journal, country of origin, study aim and study design), (b) characteristics of the intervention (setting, type of peer support, purpose of peer support intervention), (c) participant characteristics (age, gender, presenting mental health problems), (d) mental health outcomes (outcome measures and reported changes) and (e) peer worker characteristics (role, characteristics, training and supervision).

To address the question on implementation, qualitative studies were coded using the domains of the CFIR (<https://cfirguide.org/>; Damschroder et al., 2009). The CFIR is a meta-theoretical framework consisting of five domains that influence whether or not an intervention is successfully implemented: *Intervention Characteristics*, *Outer Setting* (features of the external context or environment that might influence implementation e.g. external policies and incentives), *Inner Setting* (features of the implementing organisations that might influence implementation e.g. organisation culture, leadership engagement, available resources), *Characteristics of Individuals* and *Implementation Processes*. The CFIR has been used widely in implementation science studies across a range of health service research contexts and has previously been used in reviews examining implementation of peer support interventions in adult settings (Mutschler et al., 2022).

Using this framework, the first reviewer (XX) read the articles discussing implementation and highlighted the phrases and sections that mentioned this process. The highlighted sections were then grouped together by common themes and organised into the appropriate construct within the CFIR framework. The second reviewer (XX) also reviewed the final themes to ensure that themes corresponded with the appropriate domains (Figure 1).

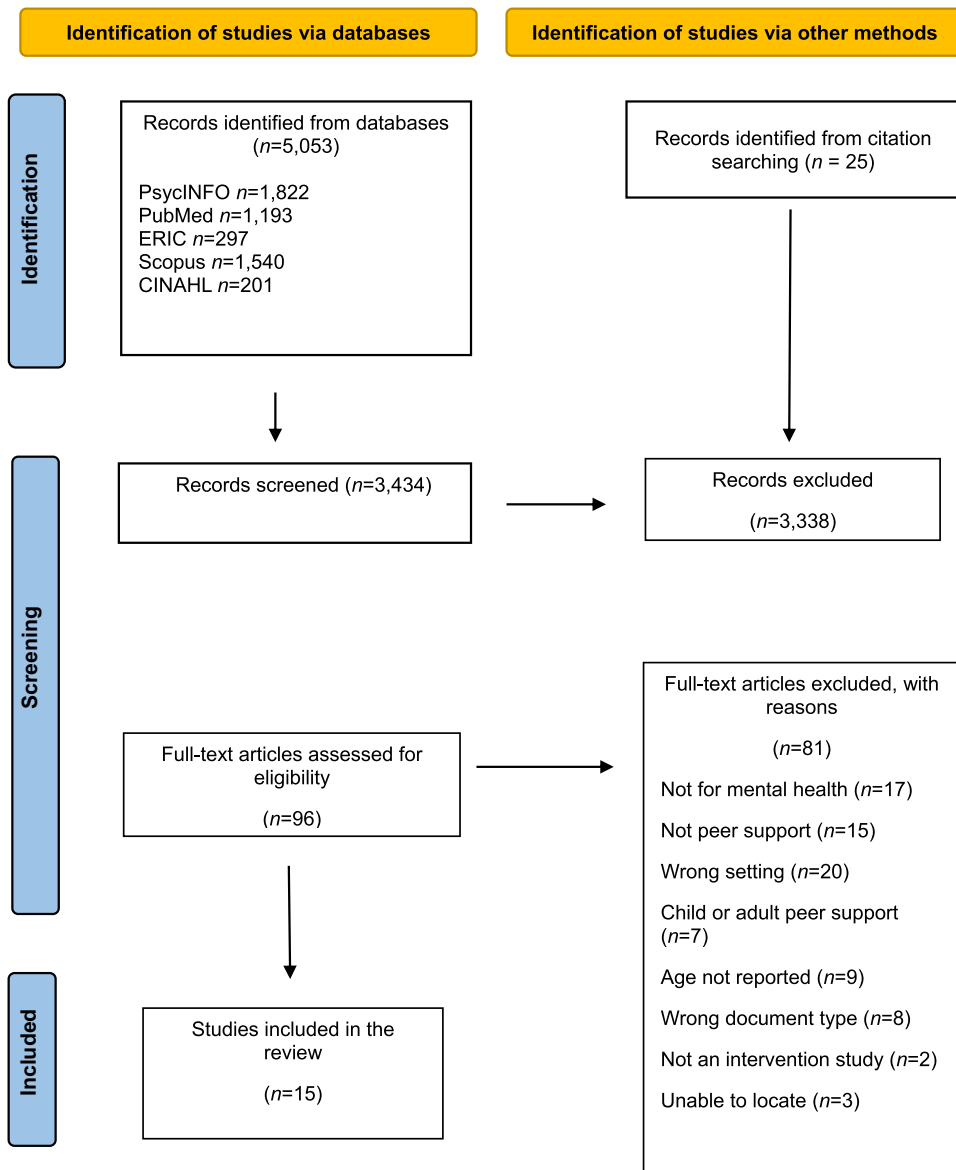


FIGURE 1 Prisma flowchart.

3 | RESULTS

3.1 | General overview

A total of fifteen studies were included for analysis in the final review (Table 1). All of the studies were published between 2011 and 2022, with ten studies (66%) published after 2020. All included studies were conducted in high income countries; five in Australia (1, 6, 10, 11, 12), four in the United States (2, 5, 8, 14), three in Canada (4, 7, 15), two in the United Kingdom (3, 13) and one in the Netherlands (9). The study methodology ranged across studies, five were randomised control trials (2, 5, 6, 7, 14), one was a non-randomised trial (10), two were cohort studies

TABLE 1 Study characteristics.

#	Study	Country	Study aim	Study design	Setting	Type of peer support	PSW role	Sample demographics	Gender
1	Alvarez-Jimenez et al. (2020)	Australia	To examine feasibility, acceptability and safety of an online mental health support programme, consisting of counselling and an online peer discussion forum.	Uncontrolled single group design	Online youth mental health service	MOST+Internet Support Group	- Peer moderation - Trained PSW	73 young people between 16–25, with concerns about their mental health. M = 19.1 SD = 2.3	Binary measure: 77% female
2	Bautista et al. (2022)	USA	To examine the effectiveness and acceptability of an online therapy programme (including peer support) compared to treatment as usual.	RCT	University	Peer delivered service	- One to one support - Trained PSW	35 undergraduate students between 19 and 24, with social anxiety. (n = 20 immediate treatment) (n = 15 waitlist) M = 21.9 SD = 4.8	Binary measure: 71.4% female
3	Byrom (2018)	UK	To identify acceptability and impact of a peer support intervention for student depression.	Cohort	University	StudentMinds Self-help group	- Group facilitation - Trained PSW	65 undergraduate students, majority of who reported mental health difficulties. M = 20.1 SD = 2.7	Non-binary measure: 70% female, 14% non-binary
4	Coulombe et al. (2020)	Canada	To evaluate effectiveness of DBT intervention with peer support component for young people aged 16–29.	Cohort	IYS	Stella's Place Self-help group	- Group facilitation with clinician - Trained PSW	76 young people experiencing mental health difficulties. M = 24.7	Binary measure: 68.3% female

(Continues)

TABLE 1 (Continued)

#	Study	Country	Study aim	Study design	Setting	Type of peer support	PSW role	Sample demographics	Gender
5	Conley et al. (2020)	USA	To test the effectiveness of the Honest, Open & Proud -College intervention in reducing stigma in disclosing mental illness.	RCT	University	Honest Open Proud Self-help group	- Group facilitation - Trained PSW	118 undergraduate or postgraduate students experiencing depression, anxiety or receiving counselling or medication. (n = 63 intervention) (n = 55 control)	Binary measure: 82% female
6	Ellis et al. (2011)	Australia	To examine the effects of an online cognitive behaviour therapy (CBT) programme compared with an online support group in decreasing symptoms of depression and anxiety, and improving dysfunctional thoughts, social support, and CBT literacy in young adults.	RCT	University	Moodgym Internet Support Group	- Peer moderation	39 between 18 and 25 undergraduate students experiencing depression or anxiety. (n = 13 control) (n = 13 CBT therapy) (n = 13 peer support) M = 19.7 SD = 1.6	Binary measure: 77% female
7	Grégoire et al. (2022)	Canada	To assess effectiveness of a peer led intervention for university students.	RCT	University	Peer delivered service	- One to one support - Trained PSW	107 university students experiencing mental health difficulties. (n = 54 intervention) (n = 53 control group)	Binary measure: 76.6%

TABLE 1 (Continued)

#	Study	Country	Study aim	Study design	Setting	Type of peer support	PSW role	Sample demographics	Gender
Age range = 17–54 M = 26, SD = 6.06 Median = 24									
8	Kerner et al. (2021)	USA	To examine the utilisation of a metropolitan peer-supported youth hotline between 2010 and 2016.	Longitudinal/Descriptive	IYS	Peer delivered service	- One to one support - Trained PSW	65,781 adolescents who accessed the hotline for mental health difficulties. Age range = 13–19	Non-binary measure: 2010: 67.47% female 2016: 72.63% female 2010: 0.10% trans-gender 2016: 0.98% trans-gender
9	Leijdesdorff et al. (2022)	The Netherlands	To provide a descriptive overview of peer worker and recipient characteristics attending a Dutch peer support service for young people aged 12–25.	Descriptive	IYS	@ease Peer operated service	- One to one support - Trained PSW	291 young people experiencing mental health difficulties aged 10–55 M = 21.6 SD = 4.2	Non-binary measure: 65% female 2% other non-binary/trans-gender)
10	Simmons et al. (2017) ^a	Australia	To evaluate an intervention implemented in an Australian youth mental health service that utilised peer workers to promote	Non-randomised control trial	IYS	The Choice Project Peer employees	- One to one support - Trained PSW	229 young people between 16 and 25 attending a youth mental health service for a mental health difficulty. (n = 149 intervention)	Binary Measure: 63% female

(Continues)

TABLE 1 (Continued)

#	Study	Country	Study aim	Study design	Setting	Type of peer support	PSW role	Sample demographics	Gender
11	Simmons et al. (2020) ^a	Australia	shared decision making via an online tool. To explore the motivations, experiences and beliefs of the YPW over time.	Qualitative	IYS	<i>The Choice Project</i> Peer employees	- One to one support - Trained PSW	(n = 80 historical control group) M = 17.8 SD = 2.9 8 peer workers, aged 17–21 M = 19.3 SD = 1.5	Non-binary measure: 4 female 3 male 1 transgender
12	Simmons et al. (2018) ^a	Australia	To describe the development and implementation of the CHOICE peer support project for young people aged 12–25.	Descriptive	IYS	<i>The Choice Project</i> Peer employees	- One to one support - Trained PSW	8 peer workers aged 16–25	Not reported
13	Stapley et al. (2022)	UK	To examine the acceptability and effectiveness of cross-age peer mentoring in a secondary school.	Mixed Methods	Secondary School	<i>More than Mentors</i> Peer delivered service	- One to one - Trained PSW	377 adolescents (peer mentors and mentees) aged 11–18. Mentors M = 15.7, Mentees M = 13.3	Binary measure: mentors—77.1% female Mentees—68.7% female

TABLE 1 (Continued)

#	Study	Country	Study aim	Study design	Setting	Type of peer support	PSW role	Sample demographics	Gender
14	Stice et al. (2017)	USA	To examine effectiveness of peer/clinician led support compared to educational video controls.	RCT	University	The Body Project Self-help group	- Group facilitation with clinician - Trained PSW	680 young women, experiencing body dissatisfaction. (n = 162 intervention) (n = 518 control groups) M = 22.2 SD = 7.1	Binary measure: 100% female
15	Suresh et al. (2021)	Canada	To examine acceptability of a student peer support programme.	Mixed methods	University	The Peer Support Centre Peer operated service	- One to one support - Trained PSW	804 university students, experiencing mental health problems including anxiety, low mood and stress. Age range = 18–35 (70% between 18 and 23)	Non-binary measure 66.5% female 1.7% non-binary

Abbreviations: IYS, integrated youth service; PSW, peer support worker; RCT, randomised control trial.
^aRepresents studies conducted on the same peer support programme. Simmons et al. (2017, 2018, 2020) were conducted on the Choice Project.

(3, 4), one was qualitative (11, 12), two were mixed methods (13, 15) one was longitudinal (8), two were descriptive (9, 12) and one was an uncontrolled single group design (1). The majority of study participants were attendees of a peer support intervention ($n = 12$) and three studies also included data from PSWs (11, 12, 13). All studies had a larger proportion of female to male participants. Sample sizes of studies ranged from 8 (11, 12) to 65,781 participants (8).

3.2 | Characteristics of peer support interventions in integrated primary YMH care

Thirteen peer support interventions were included in the review; three studies were conducted on the same intervention; *The Choice Project* (10, 11, 12). The most common setting for peer support interventions was within university services (2, 3, 5, 6, 7, 14, 15), followed by integrated YMH services (4, 8, 9, 10, 11, 12). One intervention was delivered through an online YMH service (1), and one was delivered in a secondary/high school in collaboration with a community mental health service (13).

A variety of interventions based on Solomon's (2004) categorisation were reported. Four studies were self-help groups (3, 4, 5, 14). Four studies involved peer delivered one-to-one support (2, 7, 8, 13). Three studies involved peer employees providing one-to-one support (10, 11, 12). Two studies examined peer operated services, also using one-to-one support (9, 15). The two remaining studies examined internet support groups (1, 6).

Nine out of the thirteen interventions were aimed at young people aged 16–25 years (1, 2, 3, 4, 5, 6, 7, 10, 11, 12, 14, 15). One intervention was aimed at adolescents aged 13–19 years (8), one was delivered to adolescents aged 11–18 years (13) and one intervention was aimed at young people aged 12–25 years, although the age range of attendees ranged from 10 to 55 years (9).

Eight of the thirteen interventions were targeted at young people experiencing any mild to moderate mental health difficulty such as depression, anxiety, and psychological distress (1, 3, 4, 6, 7, 8, 9, 10, 11, 12, 15). One intervention was aimed at young people who had concerns about their mental health and experienced self-stigma around disclosing their mental health difficulties to others (5). Two interventions targeted young people at risk of developing a mental health difficulty; attendees (secondary school students) of one interventions (13) were self-referred or referred by teachers if considered to be at risk of developing a mental health problem. Similarly, another programme (14) targeted female college students experiencing body dissatisfaction. One intervention was aimed at young people with social anxiety (2).

3.3 | RQ1: Reported measurement and changes in mental health outcomes of peer support interventions in integrated primary YMH care

Eleven studies reported the outcomes of the intervention for attendees' mental health. While some of the included studies investigated the alleviation of negative psychological states, other studies examined the effects of peer support on positive psychological outcomes. Common outcome measures included psychological wellbeing using the Warwick Edinburgh Mental Wellbeing Scale (1, 3, 7, 13), and psychological distress using the Kessler Psychological Distress Scale (1, 6), the Psychological Stress Measure (7), PSM-9 and the Perceived Stress Scale (1, 13). Depression was measured using the Patient Health Questionnaire (1, 7), the CES-D (5), and the DASS 21 (6). Anxiety was measured using the GAD 7 (5, 7) and DASS-21 (6). Other outcome measures used included resilience using the Connor-Davidson Scale (4) and the Student Resilience Scale (PSS) (13); social support using the Friendship Scale (1) and the Online Social Support Scale (6). One study used the Patient Health Questionnaire, GAD-7 and Outcome Rating Scale (ORS) to measure depression, anxiety and wellbeing of service users before attendance (15) but did not report these outcomes post intervention. They also employed the Session Rating Scale (SRS) to assess if participants felt the intervention improved their wellbeing.

Four studies reported improvements in psychological wellbeing of attendees (1, 3, 7, 15). One study (13) reported an improvement in overall mental health as measured by the Strengths and Difficulties Questionnaire; however, when scores were analysed separately no change was reported. This study also reported no change in psychological wellbeing as measured by the Warwick-Edinburgh Mental Wellbeing Scale.

Two studies reported improvements in social support (1, 6) and one reported a reduction in loneliness (1). Two studies reported decreased anxiety (6, 7) while one reported no change (5). Although one study (7) reported a reduction in depressive symptoms, three studies did not reveal a significant effect (1, 5, 6). One study reported an improvement in resilience (4), while the another reported no significant change (13). Studies aimed at specific mental difficulties, including social anxiety (2) and body dissatisfaction (14) reported improvements in outcomes measured.

Other improved outcomes related to managing mental health difficulties, including coping skills (4, 5), autonomy (1), ability to manage self-stigma (5), self-efficacy (4, 5) and decision making for mental health care (10). A full summary of mental health outcomes is provided in the supplementary material.

3.4 | RQ 2: Characteristics of PSWs in integrated primary YMH care

PSWs were noted across studies as being young people of similar age to those attending the intervention, although only six (46%) reported information on the mean age or age range. The oldest PSW was reported to be 27 years of age (7), with the youngest being 14 (8). See Table 2 for further information on ages of PSWs. Of the thirteen different interventions, six (40%) required PSWs to have a lived experience of mental health difficulties. Three studies (4, 7, 9) explicitly noted that PSWs should want and be able to communicate their experiences in an appropriate and helpful way. Five interventions did not specify if PSWs needed to have experienced mental health challenges (2, 8, 13, 14, 15). While one study (3) stated that personal experience of depression was not necessary to become a peer worker in their intervention, students with such experience were encouraged to apply.

3.5 | Training and supervision of PSWs

All interventions except one mentioned training of PSWs (6). Training varied in length from 8 h (14) to 5 days (7, 10, 11, 12). Two interventions involved completing a professional course/qualification. In the *Choice Project* (10, 11, 12), PSWs completed an initial 5-day training programme followed by mandatory training required by the governing employer. In *More than Mentors* (13), PSWs completed training with an educational charity in the United Kingdom. Training across interventions was mostly delivered by clinical staff in the YMH service, although two interventions used experienced PSWs (9, 15), while another engaged community partners/non-governmental organisations (8). The primary aim of training was to foster interpersonal skills necessary for the role, notably: active listening (3, 8, 9, 13, 15), and communication/conversation techniques such as motivational interviewing (3, 4, 8, 9, 13, 15). Training also covered boundary setting/safeguarding (3, 13) and crisis management (4, 7, 8, 15). Three interventions provided training on the principles/role of peer support (7, 9, 10, 11, 12).

Seven out of thirteen interventions provided supervision to PSWs in their role, which could come from clinical staff at the service (1, 3, 4, 7, 8, 9, 10, 11, 12, 13) or senior peer support staff (1, 3). Supervision also varied across studies. Supervision was offered individually (2, 10, 11, 12) or in a group (7, 13). In one study (8), PSWs were supervised while providing support to peers. In this one-to-one telephone support, a trained mental health professional listened in to phone calls and were available for debriefing afterwards. Three programmes had a "stepped supervision" system (3, 9, 10, 11, 12) which consisted of supervision from a range of senior staff, including a point of contact they could meet regularly, as well as project managers (3, 10, 11, 12). PSWs in one intervention (10, 11, 12) engaged in monthly supervision from an external expert in peer support. Emergency supervision was provided by a psychiatrist in a peer operated programme (8).

TABLE 2 Characteristics, training, supervision and role of peer support workers in integrated youth mental health care.

#	Author (year)	Lived experience	Age	Training	Supervision	Role
1	Alvarez-Jimenez (2020)	✓	NR	✓	✓	Moderate online form. Sharing practical advice and resources
2	Bautista et al. (2022)	NR	M = 20.5, SD = 1.29	✓	NR	Provide practical advice to supplement ICBT programme
3	Byrom (2018)	Optional	NR	✓	✓	Facilitate group sessions
4	Coulombe et al. (2020)	✓	NR	✓	NR	Facilitate group with clinician
5	Conley et al. (2020)	NR	NR	✓	NR	Facilitate group sessions
6	Ellis et al. (2011)	✓	NR	NR	NR	Moderate online discussion forum
7	Grégoire et al. (2022)	✓	M = 27.9, SD = 4.9	✓	✓	Online synchronous chat support with ACT
8	Kerner et al. (2021)	NR	Range = 14–18	✓	✓	Provide one to one support (telephone)
9	Leijdesdorff et al. (2022)	✓	Range = 18–30	✓	✓	Welcome young people to service
10, 11, 12	Simmons et al. (2017, 2018, 2020)	✓	Range = 16–25	✓	✓	Assist with treatment options
13	Stapley et al. (2022)	NR	M = 15.7, SD = 1.7	✓	✓	Mentoring (one to one)
14	Stice et al. (2017)	NR	M = 20.9, SD = 0.9	✓	NR	Facilitate training with clinician
15	Suresh et al. (2021)	NR	NR	✓	NR	Provide one to one support

Abbreviations: ACT, acceptance and commitment therapy; ICBT, inference based cognitive behavioural therapy; NR, not reported.

3.6 | Role of PSWs

The roles of PSWs varied across studies, and this was reflected in the terms used to describe them. These included *peer facilitators* (3, 5), *peer coaches* (2), *peer educators* (14), *peer mentors* (13) and *peer support providers* (15). In the internet support groups, the role of PSWs were to moderate posts to the sites (1, 6). Both sites provided opportunities for young people to share experiences and receive support from other young people experiencing similar challenges. The goal of these interventions was to foster social support and provide resources for young people to manage their mental health. In self-help groups, PSWs facilitated groups of young people experiencing mental health challenges (3, 4, 5, 14). Two of these groups (3, 5) were fully delivered by PSWs, using a manual or workbook. These groups were divided into sessions, with each meeting involving discussion of different topics and strategies for how to manage mental health. One intervention (3) involved 6 weekly information sessions to build mental health literacy, an opportunity to share self-care strategies, and to focus on behaviour. The other (5) included three main lessons and two boosters, where vignettes, role-plays, self-reflection exercises, and group discussions were used to help attendees manage self-disclosure of their mental health problems to others. The two other self-help groups (4, 14) involved PSWs delivering a group in partnership with a mental health professional.

The most common role of PSWs was to provide one-to-one support. In two programmes (9, 10, 11, 12), PSWs welcomed and provided informational support to young people attending a mental health service. Six interventions (2, 7, 8, 9, 13, 15) involved PSWs performing a listening role, where a PSW would create a space in which a young person could share their experiences of mental health difficulties. This role was described in one study as a *professional friend* (13). In one of these interventions (7), the PSWs were trained in Acceptance and Commitment Therapy (7) and used these therapeutic methods to guide their one-to-one sessions with peers. The final one-to-one role of PSWs was to supplement an inference based cognitive behavioural therapy and provide informational support about the clinician led group intervention (2).

3.7 | RQ3: Barriers and facilitators to implementing peer support in integrated primary YMH care

Three studies (11, 12, 13) provided information on barriers and facilitators to implementation of peer support. Implementation factors across four of the five domains of the Consolidated Framework for Implementation were discussed including: *Processes, Inner Setting, Individual Characteristics, and Intervention Characteristics*.

3.7.1 | Processes

All three studies discussed the importance of supporting PSWs and clinical staff before and during the process of implementation.

Planning

Role confusion was identified as a barrier to implementation across two studies (11, 12). PSWs experienced difficulties navigating their role with the existing service structure and with navigating their professional relationship with young people attending the service. This uncertainty surrounding their role meant that PSWs found it difficult to integrate themselves into their service and feel competent in their role (11, 12). In an attempt to address this, ensuring that PSWs had a clearly pre-defined role criteria beyond lived experience of mental health difficulties was viewed as essential (11).

Engaging

Training PSWs and clinical staff was a noted facilitator to implementing peer support programmes successfully. Skills learned during training included interpersonal and communication skills (11, 12) and general workplace and organisational skills (11, 12). Additionally, training prepared PSWs for dealing with difficult situations through role playing was a noted facilitator (13). Training was viewed positively across all studies, and PSWs felt that it improved their confidence in the role. However, two studies suggested that training before implementation might not always be sufficient and that issues may arise that are not covered in training (11, 12), such as dealing with unexpected issues or triggering situations.

One study also provided training for staff which detailed the purpose of peer work, the nature of the peer worker role and the differences between clinical and peer roles, as well as the value PSWs could add to the mental health system (12).

Champions

Ongoing supervision and support were noted across all studies as critical to successful implementation. Support could come from a supervisor, line manager or senior PSW working closely with the PSW. The role of these champions was to support PSWs, offer solution-focused advice and help to integrate them within clinical staff.

3.7.2 | Inner setting

Culture

Two studies (11, 12) noted that the inclusion of PSWs within an accepting setting was essential for implementation. Participants expressed concerns that PSWs may feel isolated and less valued than clinical staff within a service (12). To ensure successful integration of PSWs, positive relationships were fostered through involvement of PSWs representatives in regular meetings with clinical staff (12).

Structural characteristics

One study (12) noted that integration of peer support within pre-existing structures required significant re-organisation. This shift may be supported through working together to map out the differences between peer work, nonclinical work and clinical work and emphasising the unique and positive aspects of peer support to staff.

3.7.3 | Individual characteristics

Knowledge and beliefs about the intervention

PSWs across studies expressed positive feelings about their role. Participants in one study reported feeling that they "were making a difference" to young people (11) and that they were contributing to a youth friendly and welcoming atmosphere in their service. Similarly, mentors in another study (13) felt happy, proud, and accomplished in their role (13).

Non-peer staff also discussed beliefs about implementing peer support within YMH contexts (12). Concerns were raised around confidentiality, as clinical staff expressed discomfort that peer workers would have access to client records and may see friends or acquaintance in the service. Clinical staff also expressed concern over how they were to act around PSWs, given that some of them may have previously attended the service. These concerns were noted as a further barrier to building professional relationships between clinical staff and PSWs.

Self-Efficacy

PSWs acknowledged their unique role by emphasising how their relationship with young people differed to traditional clinical relationships, acknowledging their similarity in age, less formal demeanour and lived experience of mental health issues and recovery (11, 12). However, the same participants also expressed uncertainty surrounding their skills and the degree to which they themselves felt “recovered” (11). A lack of confidence in the role was noted as a barrier to engagement with peers and clinicians within the service.

3.7.4 | Intervention characteristics

Relative advantage

Participants in one study (12) expressed uncertainty regarding the benefits of peer support *above* what the service was already providing. As noted in the *Inner Setting* domain, integration of PSWs, meetings with clinical staff and supervisory support for staff with concerns are/were important to highlight the advantages of peer support.

Complexity

Two studies (11, 13) highlighted the complex nature of peer support. The process of developing relationships with attendees of programmes was not always straightforward, with PSWs encountering a range of obstacles including shy attendees, difficulty engaging with certain young people or feeling that they were giving the “wrong” advice. Additionally, due to the complex nature of peer support, one study (13) noted that it was not always possible to cover all the necessary topics and issues within the time allocated for sessions. Some mentors were not able to attend all sessions or felt that 30-min sessions could be lengthened. As an improvement/recommendation, some mentees suggested that the programme could be improved by increasing the frequency of sessions.

4 | DISCUSSION

The aim of this review was to provide a comprehensive synthesis of the literature surrounding peer support interventions within integrated primary YMH care, including IYSs and educational settings. The review addressed questions on the mental health outcomes and associated changes in studies of peer support interventions, the skills and roles of PSWs, and the barriers and facilitators to implementation of peer support interventions. Accordingly, this review can provide researchers, policymakers, and mental health practitioners with an overview of the literature so that evidence-informed decisions can be made when designing these interventions in the future.

This review identified 15 studies that aligned with the inclusion criteria given the paucity of evidence on this topic (Anderson & Zbukvic, 2020), this relatively low number of studies was expected. However, it is noteworthy the majority of studies identified were published after 2020, which is reflective of the growing interest in this area. Of the fifteen studies included, thirteen different programmes were identified. It should be noted that three studies were conducted on the same programme, *The Choice Project* at Orygen, Australia, by the same lead author. This aligns with the growing recognition of Australia as a global leader in innovative YMH care (Hetrick et al., 2017; Settapani et al., 2019). The remaining studies were conducted in the United States, the United Kingdom, Canada and the Netherlands, which further highlights the known disparities between primary YMH care in high income versus low to middle income countries (Babatunde et al., 2021; Patel et al., 2018). In terms of settings, almost half of the programmes were delivered through university services. This is not surprising given the increasing rates of mental health difficulties in students and the subsequent growth in innovative mental health interventions within university settings (Armstrong-Astley et al., 2022; Broglia et al., 2018; Mahon et al., 2022).

4.1 | RQ1: Measurement and reported changes in mental health outcomes in peer support interventions

As this review focused on peer support in integrated primary YMH care, which is often the first point of contact for young people experiencing mental health difficulties, the majority of interventions targeted the most commonly presented issues at these services (O'Reilly et al., 2022; Rickwood et al., 2019) including depression, anxiety and psychological distress. Measurements of these outcomes varied, although some common measures were employed such as the Warwick-Edinburgh Mental Wellbeing Scale, GAD-7 and Patient Health Questionnaire. Studies reported varied findings regarding the reduction of anxiety and depression, although four of the five studies measuring wellbeing reported improvements. In line with the adult literature (Bellamy et al., 2017; Lloyd-Evans et al., 2014; White et al., 2020) and recent reviews of peer support across youth settings (Gopalan et al., 2017; Richard et al., 2022) positive findings in peer support interventions were reported in relation to psychosocial and recovery related outcomes such as self-efficacy, social support, coping and autonomy. Interestingly, while studies aimed at general mental health difficulties reported varied findings on reductions in mental health difficulties, the two interventions related to specific difficulties (body dissatisfaction and social anxiety) reported medium to significant changes. It should be noted that peer support in these studies was used to supplement a clinician led intervention.

However, as this study was a scoping review, and did not conduct a meta-analysis, it is difficult to draw firm conclusions on the value of peer support for YMH outcomes. Further studies are needed to establish effectiveness.

4.2 | RQ 2: PSWs characteristics and roles

The studies in this review reported varying information regarding the PSWs involved in the interventions. First, although all studies utilised PSWs in the same age range as the young people they were supporting, only one-third reported their mean age. The majority of studies that did report age were one to one interventions, however they did not report whether PSWs were required to support a young person of the same age as them. For example, one intervention utilised PSWs aged 16–25. Given this large age range, and the differing experiences of adolescents and emerging adults, future research would benefit from exploring the differences in peer relationships with older PSWs compared to PSWs of the same age.

Several studies requested that PSWs to have an experience of mental health difficulties. This is encouraging given that lived experience is a core principle underlying peer support (Fava et al., 2020; Gillard, 2019). Within this, three interventions highlighted that PSWs should be able to openly discuss their experience to provide support. However, it is important to point out that several studies did not report any information on lived experience, which highlights the heterogeneity between reporting of PSWs characteristics.

With regard to professional development of PSWs, training was consistently mentioned across studies and was used to promote similar skills that have been noted in the adult literature such active listening, empathy and communication (Delman & Klodnick, 2017). One important gap in the literature, however, was the notable absence of frameworks or models in which training programmes were based. Supervision was also reported in over half of the studies. Supervisory support could come from a variety of sources, and in several studies was provided in a “stepped” structure, whereby PSWs received support from several members of senior staff. While this finding is positive, integration with service staff remains a challenge for PSWs (as reported later in this discussion). Researchers and practitioners should be mindful of this and continue to provide thorough supervision to ensure young peer feel confident in their role (de Beer et al., 2022).

Several types of peer support were reported, and included self-help groups, one to one support and internet support groups. The one-to-one interventions largely involved PSWs performing a listening role, although one intervention utilised peers to supplement a clinician-led intervention, while another involved PSWs utilising

therapeutic strategies. Many of the one-to-one interventions involved peer employees or were peer operated. Group based interventions, including internet self-help groups, were skills based where groups were structured into lessons focusing on strategies to manage mental health difficulties.

Given the varied characteristics and roles of PSWs reported in this review, our findings correspond with results from the adult literature indicating that PSWs are often delivering education or mentoring programmes rather than more traditional peer support models that align with the principles of mutuality and lived experience (Gillard, 2019; King & Simmons, 2018). One reason for this, as noted by Gillard (2019), could be the difficulty in implementing a social model of peer support within clinical settings. Peer support within YMH settings is still in its infancy and continued research will assist services in understanding the principles of the peer/PSW relationship that should be adhered to.

4.3 | RQ 3: Implementation of peer support interventions

Three studies described the facilitators and barriers to integrating peer support into mental health settings. PSWs were generally positive about their role, highlighting that their lived experience and similarity in age meant they could provide a unique type of support to young people attending the service. Consistent with evidence from adult settings (Ibrahim et al., 2020; Mutschler et al., 2022; Zeng & McNamara, 2021), providing sufficient support within the organisation was highlighted as a key process in ensuring PSWs were confident in their role; this included comprehensive training on interpersonal and workplace skills, and supervision from experienced clinical and peer support staff.

Another finding that echoed the adult literature was the importance of clinical staff in integrating PSWs within the organisation. According to Gillard (2019), peer support may not become properly embedded into services if stakeholders are unwilling to integrate it into existing practice or do not perceive it as a valuable mental health intervention. As noted in previous reviews (de Beer et al., 2022; Ibrahim et al., 2020; Mutschler et al., 2022), uncertainty surrounding how to interact with peer workers or lack of communication with them can result in role confusion or feeling "less valued" than clinical staff. To address these concerns and foster positive relationships between staff and peer workers, our findings suggest that training should extend beyond PSWs. Opportunities for clinicians to avail of training and support from senior staff, as well as opportunities for meeting with PSWs should be regularly provided.

4.4 | Strengths and limitations

To the authors' knowledge, this is the first scoping review examining the literature on peer support interventions specifically in integrated youth mental health services and educational settings. Reliable methodologies were employed throughout the review process. For example, a panel of young people were consulted to review the research questions, as recommended when conducting YMH research (Hawke et al., 2018; Watson et al., 2023); a specialist librarian assisted in the design of the search strategy; and the lead author was supervised during the search and screening process by experts in youth mental health research. This is also the first review to address the recommendations of Chinman et al. (2017) and Gillard (2019) by using an implementation framework to examine the barriers and facilitators to uptake of peer support in YMH settings.

Despite its strengths, this review is not without its limitations. It is difficult to draw firm conclusions on the results of the review due to the significant heterogeneity between the included studies. A variety of methodologies were employed, ranging from randomised controlled trials to descriptive studies. Although assessments are not a requirement of scoping reviews (Munn et al., 2018), it would have been interesting to establish the quality of studies, as this has been noted as a limitation of the youth peer support research (Anderson & Zbukvic, 2020).

Additionally, although this review aimed to address implementation of peer support, only three studies, two of which were based on the same intervention, were utilised to extract this data. Previous reviews (Ibrahim et al., 2020; Mutschler et al., 2022; Zeng & McNamara, 2021) have also extracted barriers and facilitators from non-qualitative papers by conducting thematic analysis on the method and discussion sections of papers, a method which may have been useful for this review.

This review lacks geographical diversity; all included studies were conducted in high income countries. This may have been due to the inclusion of papers published in English only, thus excluding studies from non-English speaking countries and limiting our understanding of peer support programmes in lower middle and low-income countries. Peer support is known to be used in African countries for young people living with HIV (Mpango et al., 2020; Mark et al., 2019; Wogrin et al., 2021), so it is likely that interventions exist for young people with mental health difficulties. As other reviews have noted, (de Beer et al., 2022; Simmons et al., 2023), a review focused specifically on peer support interventions in these countries would allow for greater understanding of how peer support is implemented across culturally diverse settings.

Lastly, the authors acknowledge that a larger number of studies related to peer support interventions may have been included in this review if a broader search strategy had been employed. Studies were excluded in this review if interventions were solely run by participating young people (did not involve a PSW), or if the intervention did not explicitly target young people with experiences of mental health difficulties. This may have resulted in the review missing some important information, particularly from academic settings where peer support is often utilised to foster a sense of belonging or improve wellbeing in students (Armstrong-Astley et al., 2022), regardless of mental health status.

4.5 | Implications and future research

The results of this scoping review provide important information for researchers and practitioners to consider when developing peer support interventions in the future. Firstly, this review highlights the need for more detailed reporting of peer support interventions including intervention content (i.e. what do one to one meetings with PSWs look like? How are group-based interventions structured?), as well as participants and clinicians (i.e., who are attending these interventions? What does clinician involvement look like?). Future research should also report detailed descriptions of the PWs including the length and content of training offered to them and clarity on core characteristics of PSWs (i.e. what is meant by *lived experience* and *similar age*?). More information on role requirements and the recruitment process would also be beneficial as this was not routinely reported.

Questions remain around the evaluation of peer support interventions. Although Gillard (2019), notes that illness focused effectiveness trials conflict with the “whole of life” peer support model, there is a need for more high-quality research examining its effect on YMH. Five RCT trials were included in this review and provided useful information on the effects of peer support on attendees' mental health. Future studies should continue to measure outcomes pre and post, however as noted in previous reviews such as that by King and Simmons (2018) appropriate measures examining both recovery related and mental health outcomes should be consistently used to provide meaningful comparisons. Furthermore, there is a need for more high quality research beyond effectiveness studies. Following guidelines from the Medical Research Council for developing and evaluating interventions (Skivington et al., 2021) studies relating to development, feasibility testing and process evaluations should be published. This would address the significant knowledge gap on issues surrounding intervention development and content, stakeholder acceptability and mechanisms underpinning peer support interventions.

Finally, future research should utilise qualitative methodologies to understand these interventions. This could include qualitative research to further understand the perspectives of clinicians working in YMH settings, as there is limited research in this area. Qualitative research has been conducted with mental health professionals working with adult peer support interventions (Collins et al., 2016; Kilpatrick et al., 2017) and has provided valuable

information on the challenges associated with peer support in mental health settings. Increased qualitative research will address the significant lack of implementation research in this area.

4.6 | Conclusion

To conclude, this scoping review summarised the available knowledge on key aspects of peer support interventions in integrated primary YMH care, including IYSs and educational settings. The recency of papers in this review indicates a growing research interest in peer support for YMH. The reviewed evidence demonstrates that young people are performing different roles, depending on the peer support intervention, setting and clinical needs of the service they are working in. This review offers novel insights into structural systems that influence uptake of peer support into service delivery; a synthesis of qualitative evidence showed that organisational buy-in is required to support implementation of these interventions. Key areas that require further research include more detailed reporting on intervention content and evaluations of peer support using appropriate outcome measures. Studies also need to clearly document the specific supports offered to PSWs, to assist services in developing appropriate training programmes and supervisory structures. Finally, going forward, mental health practitioners and young people should be consulted in the design and delivery of peer support interventions to ensure that they can be implemented within existing service structures while addressing young people's mental health needs.

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CONFLICT OF INTEREST STATEMENT

The authors declare no conflicts of interest.

DATA AVAILABILITY STATEMENT

The data that supports the findings of this study are available in the supplementary material of this article.

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SUPPORTING INFORMATION

Additional supporting information can be found online in the Supporting Information section at the end of this article.

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